MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 26 OF 28

MFT 725

Integrative Approaches in Therapy

14 teaching weeks · 0 practicals · 0 worked cases · 1 figures

ELECTIVES

1.71 units · Prerequisite: MFT 702, 715

> “Most experienced clinicians are integrative, whether or not they say so.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 725 — Integrative Approaches in Therapy

MSc Marriage and Family Therapy · Elective · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 702, 715

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	**Integrative Approaches in Therapy**	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Most experienced clinicians are integrative, whether or not they say so. The question is whether the integration is disciplined or accidental. This course builds a principled framework for combining models without producing incoherence, and teaches you to justify every combination.

How to use this book

Integration is a design problem, not a mood. Each week adds a dimension to a framework you will use for the rest of your career. By Week 14 you will have a written integrative framework, tested on three cases, and defended in a viva.

1. Course description

The course covers: the varieties of integration (assimilative, theoretical, sequential, pluralistic); common factors and their implications; the major dimensions of family therapy (systemic, attachment, behavioural, experiential, cognitive, narrative, biological); decision frameworks for matching model to client and phase; feedback-informed treatment; staged and stepped care; and the risks of eclecticism.

2. Learning outcomes

#	Outcome	Assessed
LO1	Distinguish the forms of integration and their assumptions	Exam
LO2	Explain common factors and use them to justify technique selection	Exam
LO3	Construct a coherent integrative formulation for a complex case	Case study
LO4	Sequence models across phases with an explicit rationale	Treatment plan
LO5	Use routine outcome monitoring and feedback to guide integration	Portfolio
LO6	Avoid incoherence and demonstrate coherence to a supervisor	Viva
LO7	Articulate a personal integrative framework	Framework paper

3. Assessment

Component	Weight	Due
Integrative case study (2,500 words)	35%	Week 8
Treatment plan with sequenced models (1,500 words)	25%	Week 11
Outcome monitoring portfolio	15%	Week 13
Framework paper and viva (2,000 words)	25%	Week 14

4. The fourteen-week plan

Week	Topic	Practical
1	Why integrate: evidence, clients, and clinicians	Integration audit of your own practice
2	Forms of integration	Mapping four forms onto a case
3	Common factors and their clinical use	Common-factors observation
4	The systemic dimension	Systemic formulation
5	The attachment dimension	Attachment-informed plan
6	The behavioural and cognitive dimensions	Skills module design
7	The experiential and narrative dimensions	Emotion and meaning work
8	**Case study due**; the biological dimension	Medication-aware plan
9	Matching, sequencing and staging	Sequence design
10	Outcome monitoring and feedback-informed treatment	ORS/SRS practice
11	**Treatment plan due**; comorbidity and complexity	Complex case lab
12	Culture, values and the ethics of integration	Values audit
13	**Portfolio due**; coherence and supervision	Coherence defence
14	**Framework paper and viva**	Assessed

Week 1 — Why integrate

Three pressures.

1. **Evidence.** No single model is superior for everything. Different models have stronger evidence for different problems (FBT for adolescent anorexia, EFT for couple distress, PMT for conduct problems, family psychoeducation for psychosis).
2. **Clients.** Real families present with multiple, interacting problems: trauma plus addiction plus couple distress plus a child's school refusal.
3. **Clinicians.** Most experienced therapists drift towards integration as their caseload diversifies.

The danger. Integration without discipline becomes eclecticism: a technique for every mood, no coherent theory of change, and a client who cannot tell what is expected of them.

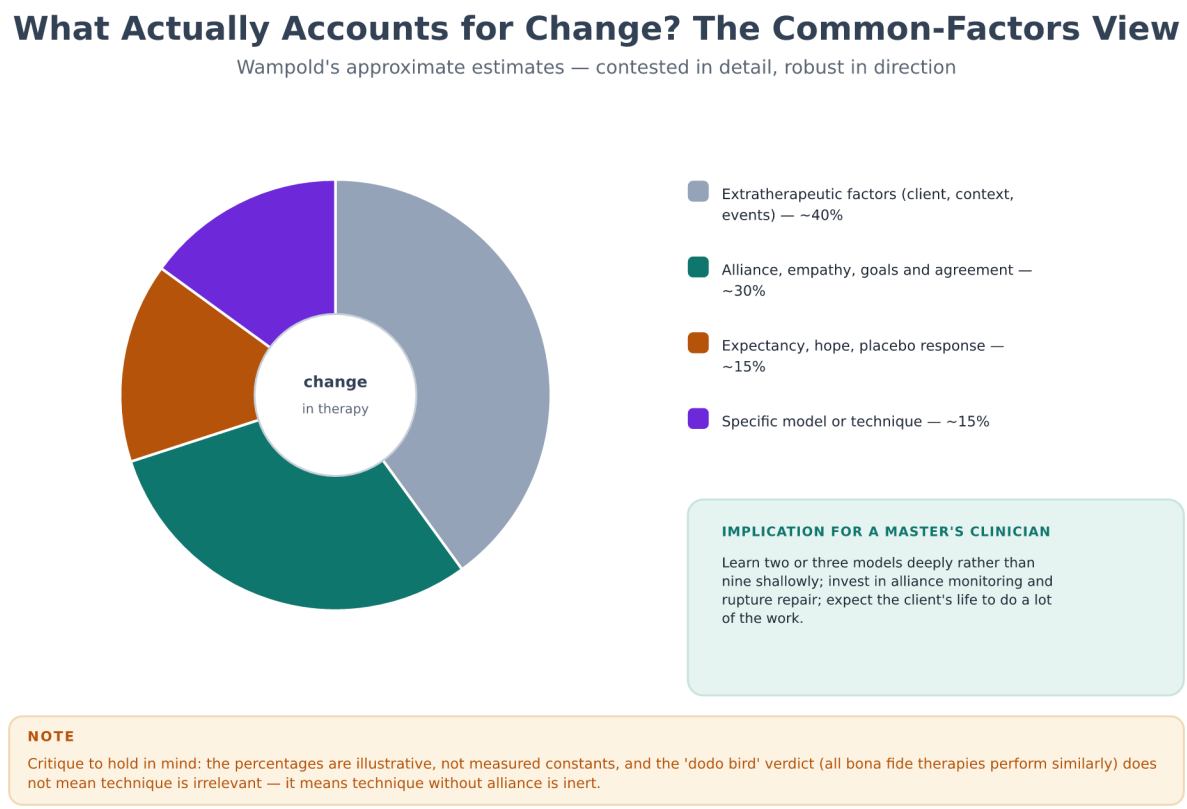


Figure 1.1 — What accounts for change: a common-factors view

> **AUDIT**
> List the models you actually use. For each, name the client problem you use it for and the theory of change you rely on. Identify any technique you use without being able to explain why.

Week 2 — Forms of integration

Form	Definition	Example	Risk
Assimilative	A home model, enriched by techniques from elsewhere	EFT therapist borrowing a behavioural task	Drift from the core theory
Theoretical	A new model synthesising several	Integrative behavioural couples therapy	Complexity, training burden
Sequential	Different models at different phases	Structural stabilisation, then EFT	Discontinuity for the family
Parallel	Different models for different subsystems	Couple work + individual trauma work	Splitting between clinicians
Pluralistic	Collaborating with the client on method	"Which approach fits you?"	Requires an informed client

Choosing a form. Assimilative integration is the most realistic for a newly qualified clinician: it keeps coherence while allowing flexibility. Sequential and parallel integration are appropriate for complexity. Pluralistic practice is an ethical stance as much as a technique.

Week 3 — Common factors

The claim. Much of psychotherapy's effect comes from factors shared across models: the alliance, agreement on goals, therapist empathy, client expectancy, and extratherapeutic factors.

The clinical use — without relativism.

1. **Monitor the alliance** in every session, with every family member.
2. **Repair ruptures** quickly; the repair is often more powerful than the original intervention.
3. **Agree goals explicitly**; disagreement about goals predicts drop-out.
4. **Engender hope** honestly (not false reassurance).
5. **Then use techniques** — which matter most for specific problems and for structuring the work.

The critique. Percentages attributed to common factors are illustrative, not measured; "all models are equal" is over-stated; and technique does matter for specific disorders. The defensible position: the alliance is necessary but not sufficient; technique without alliance is inert; alliance without technique is often insufficient.

Week 4 — The systemic dimension

What it contributes. Attention to pattern, structure, triangles, context and power. It tells you where to look and what maintains the problem.

What it lacks alone. Specific techniques for symptom change; guidance for individual emotion and trauma; and a sufficient account of biology.

Integration in practice. Systemic formulation + any intervention. You can use a behavioural task, an EFT enactment, or a narrative externalising conversation, provided the systemic formulation justifies it.

Week 5 — The attachment dimension

What it contributes. A developmental account of why bonds matter; a map of strategies under threat; a mechanism for change (corrective emotional experience, enacted in the room).

Integration with systemic work. Systemic thinking tells you the loop; attachment tells you why the loop matters so much and what each person is asking for.

Integration with behavioural work. Behavioural tasks build positive exchange; attachment work deepens the meaning of the exchange. Alone, either can be shallow.

Week 6 — The behavioural and cognitive dimensions

What they contribute. Specific, testable, teachable skills; measurable change; homework; structure; and a strong evidence base for many problems.

Integration cautions. Behavioural work can feel mechanical when the emotional bond is damaged; cognitive restructuring can be premature in acute trauma; and skills practice without meaning can become performance.

A practical sequence. Stabilise and structure (behavioural) → access emotion and meaning (experiential, attachment) → consolidate with skills and plans (behavioural, cognitive).

Week 7 — The experiential and narrative dimensions

What they contribute. Access to primary emotion; meaning-making; identity; and the language of the family's own story.

Integration. Experiential work deepens the emotional field; narrative work organises it into a story the family can hold. Together they answer "what do we feel?" and "what does it mean?".

Caution. Emotional work without containment can flood; narrative work without emotion can become intellectual. The integration is in the pacing.

Week 8 — The biological dimension

What it contributes. Sleep, medication, substances, hormonal change, illness, and the physiology of stress and trauma.

Integration. A formulation that ignores biology will fail in predictable ways: the depressed client who does not respond to therapy because of untreated hypothyroidism; the couple whose sexual problem is a medication side effect; the adolescent whose sleep deprivation mimics ADHD.

Your role. Screen, refer, coordinate, and never advise on medication.

Week 9 — Matching, sequencing and staging

Decision framework — five questions.

1. **What is the most dangerous thing here?** (Risk governs sequence.)
2. **What is the most changeable thing here?** (Start where change is possible; hope is built on early wins.)

3. **What must be in place before other work can happen?** (Safety, stabilisation, attendance, sobriety.)
4. **Which model has the best evidence for the primary problem?** (Use it, or refer.)
5. **What does this family want and believe in?** (Engagement is a precondition, not a luxury.)

Staging example. A couple with active violence, alcohol misuse, and a child's school refusal: Phase 1 — safety and sobriety (individual, specialist referral); Phase 2 — couple de-escalation and parenting plan; Phase 3 — child's return to school and family consolidation.

Week 10 — Feedback-informed treatment

The instruments. The Outcome Rating Scale (ORS) and Session Rating Scale (SRS) are brief, free, and practical: four visual analogue items each, completed every session.

How to use them.

1. Introduce them as a way of making sure the therapy is working.
2. Look at the scores before you speak.
3. Ask about a drop: "Your score fell by three points. What happened?"
4. Ask about the alliance: "Is there anything about how we work that is not fitting?"
5. Act on the answer in the session, not next month.

The evidence. Routine outcome monitoring with feedback improves outcomes and reduces deterioration, particularly for clients who are not progressing. It is one of the most robust findings in the field.

> **PORTFOLIO (15%)**
> Ten sessions with ORS/SRS data, your interpretation, and the changes you made.

Week 11 — Comorbidity and complexity

Complexity is the norm. Typical community caseloads include trauma plus substance use plus couple distress plus child behaviour plus financial strain.

Principles.

1. Formulate the whole system, not a list of diagnoses.
2. Identify the keystone: the one change that would make others possible.
3. Sequence: safety → stabilisation → skills → processing → consolidation.
4. Share the load: refer where specialist input is needed; coordinate rather than compete.
5. Review: complexity requires more frequent review, not longer sessions.

Week 12 — Culture, values and the ethics of integration

Culture shapes which models fit. An approach that prizes emotional expressiveness may feel exposing; one that prizes autonomy may feel cold; one that requires homework may be impractical.

Values shape technique choice. Your beliefs about gender, family duty, forgiveness and individual autonomy will show up in what you emphasise. Audit them.

Ethical integration. Do not use a technique you are not trained in; do not combine models to appear sophisticated; and do not use integration as a reason to avoid referring.

Week 13 — Coherence

The coherence test.

Question	If the answer is no…
Can I state the formulation in three sentences?	You do not have one yet
Does every intervention follow from the formulation?	You are improvising
Can I name the model I am using at this moment?	You are drifting
Can I predict what will happen next, and be wrong productively?	Your formulation is too vague
Would a supervisor understand my reasoning without me explaining it?	Document it

The coherence defence. Present a case to peers and answer: what are you doing, why, and how will you know whether it worked?

> **COHERENCE DEFENCE**
> Ten minutes with a case, followed by hostile questioning from peers.

Week 14 — Your integrative framework

Write 2,000 words:

1. Your home model and its theory of change.
2. The dimensions you integrate (systemic, attachment, behavioural, experiential, narrative, biological).
3. Your decision rules for matching and sequencing.
4. Your use of feedback and outcome data.
5. Your limits: what you do not do, and to whom you refer.
6. How you will know you are drifting.

Defend it in a viva.

Glossary

Assimilative integration — a home model enriched by techniques from elsewhere. **Common factors** — mechanisms shared across therapies (alliance, empathy, expectancy, goals). **Feedback-informed treatment** — using session-by-session outcome and alliance data to adjust therapy. **Keystone change** — a change that makes other changes possible. **Pluralistic practice** — collaborating with the client to choose methods. **Sequential integration** — using different models at different phases of treatment. **Stepped care** — matching intensity of intervention to need.

Self-assessment

Multiple choice

1. The main risk of undisciplined integration is: (a) cost (b) incoherence for the family (c) slow progress (d) paperwork

2. Assimilative integration means: (a) equal blending (b) a home model enriched by borrowed techniques (c) alternating models by session (d) refusing a model

3. Common factors research suggests: (a) technique is irrelevant (b) alliance matters greatly and technique matters for specific problems (c) all models are identical (d) diagnosis is useless

4. The first question in sequencing is: (a) what is most interesting (b) what is most dangerous (c) what is cheapest (d) what the therapist prefers

5. Feedback-informed treatment uses: (a) long questionnaires (b) brief session-by-session measures such as ORS/SRS (c) only diagnosis (d) supervisor ratings

6. A keystone change is: (a) the first session (b) a change that makes others possible (c) a diagnosis (d) a referral

7. The dimensional approach includes all except: (a) systemic (b) attachment (c) behavioural (d) astrological

8. Integration is unethical when it involves: (a) borrowing a technique (b) using techniques outside your competence (c) sequencing models (d) monitoring outcomes

9. Coherence is demonstrated by: (a) using many techniques (b) a formulation from which interventions follow (c) longer sessions (d) confidence

10. Complexity should lead to: (a) longer sessions (b) more frequent review and coordination (c) more diagnoses (d) abandoning formulation

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 d · 8 b · 9 b · 10 b

Short answer

1. Distinguish four forms of integration with an example of each.

2. Give three clinical uses of common-factors research.
3. State the five sequencing questions.
4. How do you use ORS/SRS data in a session?
5. What are three ethical limits on integration?

Essay (choose one, 2,000 words)

1. "Integration without a theory of change is improvisation." Discuss with a case.
2. Design an integrative treatment plan for a family with trauma, addiction and couple distress.
3. Critically evaluate feedback-informed treatment as a professional standard.

Further reading

- Lebow, J. L. (2014). Couple and Family Therapy: An Integrative Map of the Territory. APA.
- Sprenkle, D. H., Davis, S. D., & Lebow, J. L. (2009). Common Factors in Couple and Family Therapy. Guilford.
- Duncan, B. L., Miller, S. D., & Sparks, J. A. (2004). The Heroic Client. Jossey-Bass.
- Norcross, J. C., & Goldfried, M. R. (Eds.). (2019). Handbook of Psychotherapy Integration (3rd ed.). OUP.

Next: MFT 726 — Neurobiology of Relationships